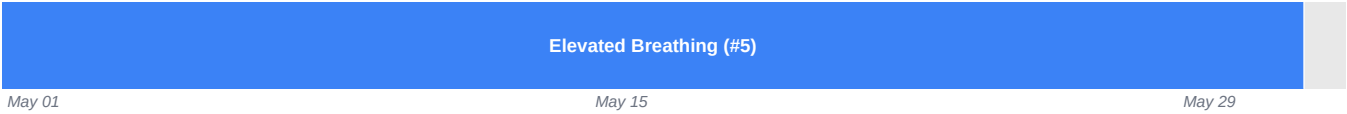


CLINICAL INTELLIGENCE TREND REPORT

RED: Provider Review Recommended

Patient ID: Wheeler Period: May 1 to June 1, 2026 Report Date: June 2, 2026 Coverage: 743/745h (100%)
Decision-support summary derived from longitudinal vital sign trends; interpret in clinical context.

Patient clinical status over 32 days from May 01 to Jun 01 ■ HR ■ Breathing



Episodic Events	Total Hours	Highest Burden	Episodes/day	Coverage
77	278h	Elevated Breathing	2	100%

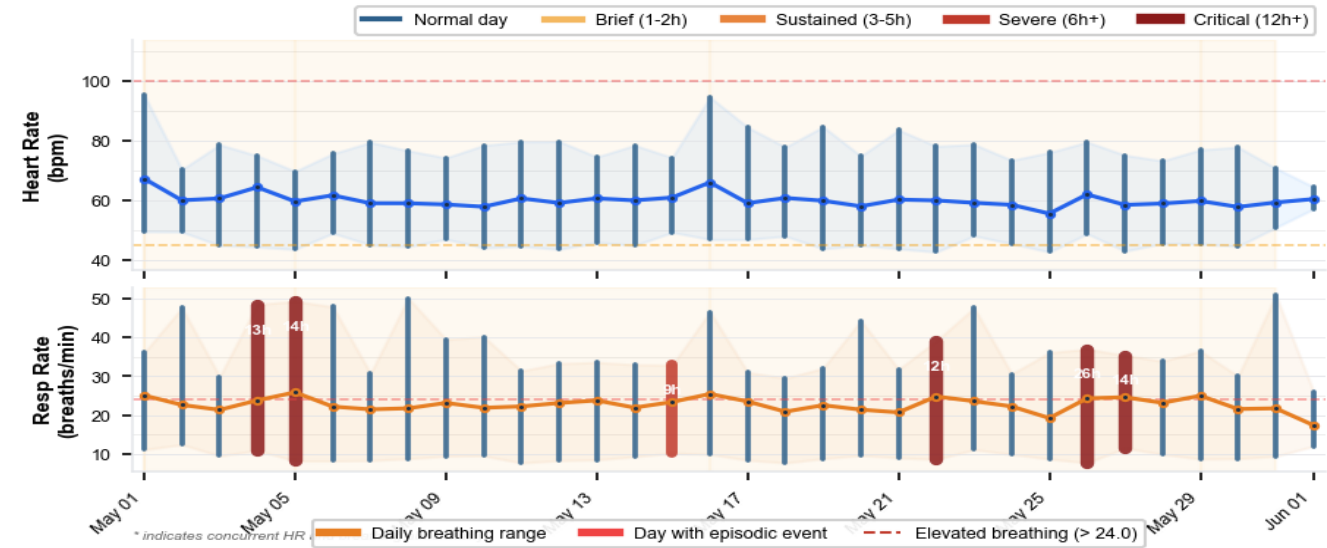
Major Findings: Sustained elevated breathing (avg 26, peak 51)

#	Episode	Min/Max	Total Hours	Longest Continuous	Episodes/d ay	Average	Date
1	High Breathing	34 brpm	4h	4h	1	31 brpm	May 01
2	High Breathing	36 brpm	4h	3h	2	31 brpm	May 16
3	High Breathing	49 brpm	1h	1h	1	32 brpm	May 05
4	High Breathing	36 brpm	1h	1h	1	32 brpm	May 29
5	Elevated Breathing	51 brpm	268h	14h	2	26 brpm	May 01 to May 31

Daily recorded hours declined from 24.0 to 1.0 (96%) over the monitoring period.

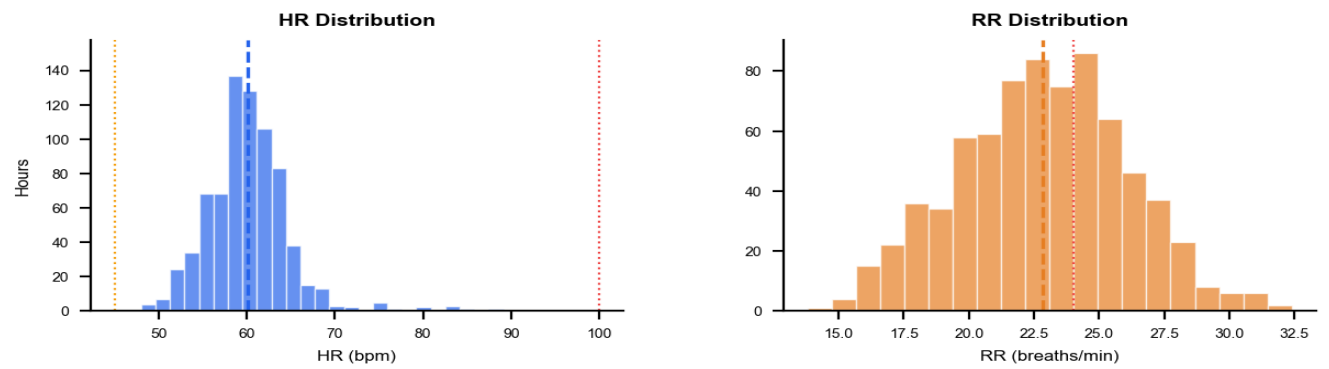
Clinical Pattern Observations:

- 1. **Sustained finding:** 14h continuous Elevated Breathing on May 05.
- 2. **Breathing variability:** RR spread 10 brpm (17 to 27).
- 3. **Clustered pattern:** Episodic events on 6 of 32 days (18%).

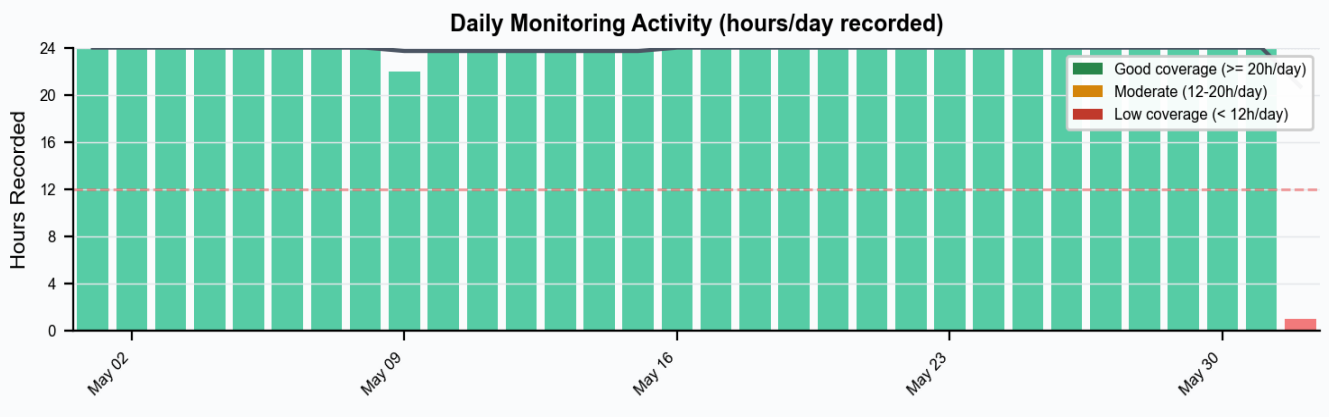


Red bars indicate days with detected episodic events.

Vital Sign Distribution



Monitoring Activity



Bars indicate monitoring hours/day. Red threshold line indicates clinical monitoring baseline.

■ HR episode ■ RR episode ■ Recorded, no episode ■ No data

Clinical Alerting Thresholds

Very Low HR < 40 bpm

Low HR < 45 bpm

Elevated HR > 95 bpm

High HR > 100 bpm

Very High HR > 110 bpm

Elevated Breathing > 24 brpm

High Breathing > 30 brpm

Very High Breathing > 40 brpm

Episodic event: threshold exceeded continuously for ≥ 1 hour. Episodes separated by ≤ 1 hour of normal vitals are counted as the same event. Brief threshold crossings that do not sustain are not flagged.